

Northgate Radiology Associates — Thoracic Imaging

CT PULMONARY ANGIOGRAPHY (CTPA) REPORT

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY — CONTAINS NO REAL PATIENT DATA

Patient: Ashley M. Reyes	DOB: 08/09/1989	MRN: 9032561
Date: November 1, 2024 at 14:32	Provider: Dr. Karen Hoffmann, MD — Diagnostic Radiology	

CLINICAL INDICATION

Ordering physician: Dr. P. Mathews, MD (Emergency Medicine). Clinical concern for pulmonary embolism. Patient is a 35-year-old female presenting with 3 days of right-sided pleuritic chest pain, mild dyspnea, and tachycardia (HR 108). Wells PE score: 4.5 (moderate probability). D-dimer: 1.82 mcg/mL (elevated). Oral contraceptive use noted.

TECHNIQUE

Study:	CT Pulmonary Angiography (CTPA) with IV contrast	Contrast:	Isovue 370, 100 mL IV — bolus tracking technique
kVp / mAs:	100 kVp / automated mA (SmartmA)	Slice Thickness:	1.25 mm reconstructions
Radiation DLP:	312 mGy-cm		

FINDINGS

PULMONARY VASCULATURE: The main pulmonary artery and bilateral central, lobar, segmental, and subsegmental pulmonary arteries opacify normally with contrast. No intraluminal filling defects are identified. No evidence of pulmonary embolism.

LUNGS AND AIRWAYS: Lungs are clear bilaterally without consolidation, ground glass opacity, or pleural effusion of significance. There are trace bilateral pleural effusions (estimated <30 mL each), likely reactive. There is a solitary, well-circumscribed, non-calcified pulmonary nodule in the right lower lobe measuring 6mm in greatest dimension. No satellite nodules. No mediastinal or hilar lymphadenopathy. Visualized airways patent without endobronchial lesion.

HEART AND MEDIASTINUM: Heart size normal. No pericardial effusion. Mediastinal contour unremarkable. No aortic abnormality on limited non-gated assessment.

BONES AND SOFT TISSUES: No suspicious osseous lesion. No significant subcutaneous abnormality.

IMPRESSION

1. NO PULMONARY EMBOLISM identified on CT pulmonary angiography.
2. Solitary 6mm right lower lobe pulmonary nodule. Given patient age (35 years) and no documented high-risk smoking history, this is most likely benign. Per Fleischner Society 2017 Guidelines for incidental pulmonary nodules in low-risk individuals: recommend CT chest follow-up at 12 months to confirm stability.
3. Trace bilateral pleural effusions, likely reactive in context of clinical presentation — no intervention required.

Report finalized by Dr. Karen Hoffmann, MD, Board-Certified Diagnostic Radiologist, November 1, 2024 at 16:18.

This document contains entirely synthetic, AI-generated data for demonstration purposes only. No real patient, provider, or institutional information is represented.